



CIMETIDINE-200 TABLET

COMPOSITION:

Each tablet contains
Cimetidine

200mg

Contains Quinoline Yellow and Brilliant Blue as colourant.

PRESENTATION:

Light green, biconvex, round, film-coated tablet, 9.8mm in diameter with a single score-line on one side and marked KOTRA logo on the other side.

INDICATIONS:

Treatment of active duodenal, benign gastric, stomal and recurrent ulceration, persistent dyspeptic symptoms with or without ulceration. NSAID - induced lesions (ulcers, erosions) and symptoms, gastro-esophageal reflux disease (including conditions ranging from heartburn to peptic esophagitis), Zollinger-Ellison syndrome and other cases of high gastric secretion, in pancreatic insufficiency as an adjunct to enzyme supplement therapy.

PHARMACOLOGY:

Mechanism of Action:

Cimetidine inhibits both the stimulated and basal secretion of gastric acid and reduces pepsin output. It competitively inhibits the action of histamine at the histamine H₂-receptor of the parietal cells and thus is a histamine H₂ antagonist. In addition to its antisecretory effects, cimetidine also has cytoprotective properties and therefore has a beneficial effect in maintaining the integrity of the gastric mucosal barrier.

Pharmacokinetics:

Cimetidine is rapidly absorbed after oral administration. Its half-life is approximately 2 hours and the principal route of excretion is through the urine.

DOSAGE AND ADMINISTRATION:

For oral administration only.

a) Active ulcer: in active duodenal benign gastric and stomal ulceration, 800mg as a single dose at bedtime or 400mg twice daily with breakfast and at bedtime. Treatment should be continued for at least 4 weeks, even if symptomatic relief has been achieved in

shorter time.

b) Prevention of recurrent ulcer: In patients with recurrent duodenal and benign gastric ulceration, maintenance dose of 400mg at bedtime for as long as the physician deems necessary to prevent a relapse.

c) Persistent dyspeptic symptoms with or without ulceration: 200mg twice daily after breakfast and at bedtime. Individual courses of treatment should not exceed 4 weeks. If there is no clinical response or if symptoms reoccur rapidly or repeatedly, review the diagnosis.

d) Treatment of NSAID - induced lesion (ulcers, erosions) and symptoms: 800mg daily, either as 800mg at bedtime or 400mg twice daily at breakfast and at bedtime. Treatment should be given for at least 4 weeks. Maintenance treatment is 400mg at bedtime.

e) Gastro-esophageal reflux disease (including conditions ranging from heartburn to peptic esophagitis): 0.8 - 1.6 g/day, depending on the degree of severity of the condition. A single bedtime dose of 800mg or 400mg twice daily in the morning and at bedtime. Severe cases may require up to 1.6g/day given in divided doses. Treatment should be taken for up to 12 weeks. 200mg 4 times a day with meals and at bedtime has been show effective in heartburn.

f) In Zollinger-Ellison Syndrome and other cases of high gastric secretion: Doses should be adjusted to individual patient needs and should continue as long as clinically indicated. Doses up to 12g/day have been used.

g) In pancreatic insufficiency as an adjunct to enzyme supplement therapy: 0.8 - 1.6g/day, depending upon individual response, in 4 divided doses, one to one and half hours before meals.

Dosage should be reduced in patients with impaired renal function according to creatinine clearance. The following dosage are suggested:

Creatinine clearance

0 - 15 ml/min
15 - 30ml/min
30 - 50ml/min
Over 50ml/min

Dosage

200mg twice a day
200mg three times a day
200mg four times a day
Normal dosage

Elderly:

The normal adult dosage may be used unless renal function is markedly impaired.

Children:

In children more than one year old, 25-30mg/kg body weight per day in divided doses.

CONTRAINDICATION:

Patients with known hypersensitivity to the drug.

PRECAUTION:

The safety of prolonged use is not, however fully established and care should be taken to observe periodically patients given prolonged treatment. Cimetidine

treatment can mask the symptoms and allow transient healing of gastric cancer. The potential delay in diagnosis should particularly be borne in mind in patients of middle age and over with new or recently changed dyspeptic symptoms. Care should be taken for patient with a history of peptic ulcer, particularly the elderly, being treated with cimetidine and non-steroidal anti-inflammatory agent are observed regularly. It should be avoided during lactation unless essential. In patients in drug treatment or with illness that could cause falls in blood cell count, the possibility that H₂ receptor antagonism could potentiate this effect should be borne in mind. Exclude malignancy in gastric ulcer. Use in pregnancy should be avoided unless essential for benefit. Dosage to be reduced in patients with severely impaired renal function.

SIDE EFFECTS:

Agranulocytosis, headache, myalgia, arthralgia, sinus bradycardia, tachycardia, heart block and reversible impotence. Diarrhoea, tiredness, dizziness, rash in small number of patients. Gynaecomastia, rarely. Decreased white blood cell counts including agranulocytosis mostly in patients also receiving drug known to reduce blood counts. Very rare cases of thrombocytopenia and aplastic anaemia. A few cases of reversible confusion, mostly in elderly and/or severely ill patients. Rare cases of hepatitis, pancreatitis and interstitial nephritis.

DRUG INTERACTIONS:

Cimetidine apparently through an effect on certain microsomal enzymes system, has been reported to reduce the hepatic metabolism of warfarin-type anticoagulant, phenytoin, propranolol, nifedipine, chlordiazepoxide, diazepam, certain tricyclic antidepressant, lidocaine, theophylline and metronidazole, thereby delaying elimination and increasing blood levels of these drugs. Clinically significant effects have been reported with the warfarin anticoagulants; therefore, close monitoring of prothrombin time is recommended, and adjustment of the anticoagulant dose may be necessary when cimetidine is administered concomitantly. Interaction with phenytoin, lidocaine and theophylline has also been reported to produce adverse clinical effects.

OVERDOSAGE & TREATMENT:

Acute overdosage of up to 20gm has been reported several times with no significant ill effects. Induction of vomiting and/or gastric lavage may be employed together with symptomatic and supportive therapy. Animal studies with extremely high dosages indicate that artificial respiration may be of value.

STORAGE:

Keep container well closed. Store below 30°C. Protect from light.

KEEP OUT OF REACH OF CHILDREN JAUHI DARI KANAK-KANAK

PACK QUANTITIES:

Available in blister pack of 10 x 10's and 100 x 10's.

Further information can be obtained from pharmacist, physician or the manufacturer.

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Manufactured By & Product Registration Holder:



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